



- Insomnia is suspected
- Take history
- Consider assessment using the [insomnia severity index](#)
- Confirm new diagnosis of insomnia

Consider other conditions/comorbidities/triggers

e.g. obstructive sleep apnoea, Circadian rhythm disorders, restless legs, chronic pain, COPD, heart failure, GORD, menopause, psychological disorders e.g. depression, anxiety, stress, alcohol, medications, occupational at-risk group e.g. professional - use of machinery, drivers.

Treat condition/comorbidity/cause such as anxiety/depression.

Consider referral e.g. sleep clinic, neurology, see page 2

See: (<https://www.coventryrugbygateway.nhs.uk/pages/sleep-clinic/>)

Offer advice on sleep hygiene including use of [sleep diary](#) - click image for patient resources:



Insomnia
Find out about insomnia, including why it might happen and how to treat it.
www.nhs.uk



Sleep problems - Every Mind Matters
Learn about sleep problems, including what it means to be sleep deprived or have insomnia. Plus find out how sleep...
www.nhs.uk



How to sleep better
This guide offers tips on how to sleep better - looking at improving the quality of your sleep, what causes sleep...
www.mentalhealth.org.uk

If sleep hygiene measures ineffective and significant distress and daytime symptoms despite management of other factors, assess **duration** of insomnia

Short-term insomnia

(symptoms less than 3 months)

Likely to resolve soon
(e.g. bereavement, short-term stressor)?

No

Consider referral to digital CBTi or other local services as appropriate
[see page 2](#)

Yes

Consider short course of Z drug
(course length: 3 to 7 days) –
[see page 2](#)

Review within 2 – 4 weeks (dependant on the clinical situation) if symptoms have not improved, reassess including duration of insomnia, consider alternative diagnosis and need for referral

Consider **daridorexant** for chronic insomnia whose daytime functioning is considerably affected, only if CBTi has been tried but not worked, or is unavailable or unsuitable – see page 2

Long-term (chronic) insomnia

(symptoms on at least 3 nights per week for ≥ 3 months AND daytime functioning considerably affected)

Does the patient also present with **severe symptoms / acute exacerbation**?

Yes

Consider short course of Z drug:
Course length: 3 to 7 days
(may be considered as a temporary adjunct alongside digital CBTi)
[see page 2](#)

For all patients, offer digital CBTi or other local services as appropriate - [see page 2](#)

Review if ongoing symptoms at end of course or sooner

No

Has digital CBTi / other local services been *either*:
a) tried but not worked, or b) is unsuitable?, or c) refused by patient?

Yes

Is patient aged 55 years or over?

No

Yes

Consider **melatonin MR 2mg** for the short-term treatment of primary persistent insomnia characterised by poor quality of sleep in patients who are aged 55 or over – [see page 2](#)

Treatment failure

Review within 2 – 4 weeks (dependant on the clinical situation) if symptoms have not improved.
Reassess, consider alternative diagnosis and need for referral If no improvement within 13 weeks (melatonin), 12 weeks (daridorexant); stop the medication
[see page 2](#)

References

- 1.NICE TA 922. Available at: <https://www.nice.org.uk/guidance/ta922>
- 2.NICE Clinical Knowledge Summary Insomnia Last revised June 2024. Available at: <https://cks.nice.org.uk/topics/insomnia/>
- 3.SPCs. Available at: www.medicines.org.uk
- 4.BNF. Available at: <https://bnf.nice.org.uk/>
- 5.Dictionary of medicines and devices (dm+d) browser. Available at: <https://dmd-browser.nhsbsa.nhs.uk/>
- 5.NICE MTG 70: Sleepio. Available at: <https://www.nice.org.uk/guidance/mtg70>
- 6.Sleepstation. Available at: https://www.sleepstation.org.uk/nhs_options/#gp-lookup
- 7.BAP Sleep guidelines. Available at: https://www.bap.org.uk/pdfs/BAP_Guidelines-Sleep.pdf
8. PrescQIPP. Bulletin 352. Insomnia. Available at: <https://www.prescipp.info/our-resources/bulletins/bulletin-352-insomnia/> (login required to access)
- 9.Nie, T., Blair, H.A. Daridorexant in Insomnia Disorder: A Profile of Its Use. CNS Drugs 37, 267–274 (2023). Available at: <https://link.springer.com/article/10.1007/s40263-023-00987-9#Sec5>
10. Riemann, D., Espie, C. A., Altena, E., et al. (2023). The European Insomnia Guideline: An update on the diagnosis and treatment of insomnia 2023. *Journal of Sleep Research*, 32(6), e14035. Available at: <https://doi.org/10.1111/jsr.14035>

Referral for Digital CBT for insomnia (CBTi):

1. **Sleepio**® is a digital CBTi-based treatment recommended by the National Institute for Health and Care Excellence (NICE May 2022 ([MTG70](#))).
It is free only in certain areas in England on the NHS, patients can access it online through the website and their NHS login.
At the time of publication, Sleepio® is **also offered free to access for patients with a cancer diagnosis across England**.
See: <https://www.macmillan.org.uk/cancer-information-and-support/awake-and-cant-sleep>
2. **Sleepstation**® is available depending on local funding by practice/PCN.
3. **Sleepful.org.uk** is a free evidence based self-help CBTi programme, if a patient not able to access the above digital CBTi programmes, consider recommending,

Referral for Structured Psychological Therapies including CBT:

NHS Talking Therapies [Home](#) | [Coventry, Warks & Solihull Talking Therapies \(covwarkpt.nhs.uk\)](#) (previous name IAPT)

Provides structured psychological therapies for people aged 16+ who are registered with a GP and are experiencing mild to moderate anxiety related disorders/depression (may include insomnia as a component) and likely to benefit from brief psychological therapy which may include a CBT approach. Patients can self-refer. Individual sessions are 30 to 50 minutes and can be done over the telephone, video, or face to face. Group sessions will usually last for 2 hours over a 6 - 8 week period. **Note: face-to-face CBTi is a unique, stand-alone form of CBT specific to treating insomnia and not currently funded locally.**

Other Mental Health Services:

If the patient also has an underlying mental health condition(s), consider referral to **Secondary Care Community Mental Health and Wellbeing Teams (CMHWT)** or the **Primary Care Psychology Team**. See: <https://www.coventryrugbygateway.nhs.uk/>

Pharmacotherapies Prescribing Information: See BNF and product SPCs for full prescribing information

Z drugs (short term use only): non-benzodiazepine hypnotics, developed to overcome some of the adverse effects of benzodiazepines (e.g. next-day sedation, dependence, withdrawal). Like benzodiazepines, they are GABA receptor agonists.

Dosage: Prescribe for short-term (preferably 3-7 days in duration, not exceeding two weeks) on acute Rx - use only when non-drug measures have failed and insomnia is severe, disabling or causing extreme distress

Zopiclone (1st line) choice) dose 7.5mg at night (3.75mg for elderly) **Zolpidem (2nd line choice) dose 10mg at night (5mg for elderly)**

Mechanism of Action: Gaba receptor agonist

Hepatic impairment: Avoid in severe, caution in mild to moderate.

Cautions: Risks include falls (consider risk if elderly), cognitive impairment, tolerance, dependence, and withdrawal symptoms. chronic pulmonary insufficiency (increased risk of respiratory depression); depression; elderly; history of alcohol abuse; history of drug abuse; muscle weakness; psychiatric illness. See: [NICE CKS on Benzod/Z Drug withdrawal](#)

Benzod/Z Drug withdrawal

Common side effects: Abdominal pain; anterograde amnesia; anxiety; back pain; diarrhoea; dizziness; fatigue; hallucination; headache; increased risk of infection; nausea; sleep disorders; vomiting, dry mouth, bitter taste

Cost (Drug Tariff): Up to £1.54/28 tabs (Drug Tariff Dec 2024)

Melatonin Modified Release:

Dosage: 2mg od (1–2 hours before bedtime and after food)

At initiation: Complete the insomnia scoring index, agree clear treatment goals/outcomes to continue. Recommended initial duration of treatment is 3 weeks (acute Rx) for **up to 13 weeks total duration**.

Continuation: Use insomnia scoring index to support with efficacy assessment. Stop if not reached agreed goals/ outcomes. If there is a response to treatment, continue for a further 10 weeks only.

Mechanism of Action: Regulation of Circadian Rhythm via MT1 and MT2 receptor activity

Hepatic impairment: Avoid

Cautions: May cause drowsiness, autoimmune disease; susceptibility to seizures (risk of increased seizure frequency)

Common side effects : Arthralgia; increased risk of infection; pain, falls, fractures.

Cost (Drug Tariff): £3.69/ 30 tabs (Drug Tariff Dec 2024)

Daridorexant:

Dosage: 50 mg oral tablet once daily (as acute Rx), dose to be taken within 30 minutes before bedtime, alternatively 25 mg once daily in those with moderate hepatic dysfunction or when taking moderate CYP3A4 inhibitors, ciclosporin, ciprofloxacin

At initiation: Patients should not be co-prescribed other hypnotics for a month prior to initiation.

Complete the insomnia scoring index, agree clear treatment goals/outcomes to continue.

Continuation: Assess response within the first month, **with a further assessment within 3 months**. Use insomnia scoring index to support with efficacy assessment. Stop if not reached agreed goals/ outcomes. Down-titration not required.

If daridorexant is effective and continued, prescribe on repeat Rx. Reassess at regular intervals (minimum 6 monthly). **Treatment should be as short as possible; efficacy beyond one year is uncertain.**

Mechanism of Action: Dual Orexin Receptor Antagonist thereby decreasing wakefulness.

Hepatic impairment: Avoid in severe impairment.

Cautions include: Depression (worsening of symptoms including suicidal ideation); elderly (limited information available in patients older than 75 years); psychiatric illness (suicidal ideation reported in those with pre-existing disorders)

Common side effects: Dizziness, headache, nausea, fatigue, drowsiness may persist next day, especially when starting.

Cost (Drug Tariff): £42.00/ 30 tabs (Drug Tariff Dec 2024)